

Death Pronouncement and Postmortem Care

Policy Number	POL-PC-064	Owner / Department	Medical Staff / Nursing
Effective Date	2023-04-15	Version	2.1
Last Revised	2026-02-26	Next Review	2028-02-26
Approved By	Medical Executive Committee	Applies To	All inpatient units, the Emergency Department, and procedural areas; all licensed providers and nursing staff

1. Purpose

To ensure a respectful, legally sound process for pronouncing death, caring for the body, honoring the family's wishes, and meeting organ-donation and death-reporting obligations, consistent with CMS Conditions of Participation on patient rights, organ/tissue donation, and autopsy.

2. Scope

Applies to the death of any patient within the facility. Pronouncement is performed by a provider authorized under state law and medical-staff privileges.

3. Definitions

- **Pronouncement of death** — the clinical determination and documentation that death has occurred, by an authorized provider.
- **Medical examiner / coroner case** — a death that by law must be reported (e.g., unexpected, unexplained, violent, or within a defined period of admission or a procedure).
- **OPO** — the federally designated Organ Procurement Organization that must be notified of every death and imminent death to evaluate donation suitability.

4. Procedure — Pronouncement and Immediate Notifications

IMPORTANT — Every death and imminent death must be reported to the designated Organ Procurement Organization in a timely manner as required by CMS, before any approach to the family about donation. Do not initiate the donation conversation with the family until the OPO has been notified and directs or collaborates on the approach.

1. An authorized provider assesses the patient and pronounces death, recording the date, time, and absence of vital signs.
2. Notify the attending provider (if not the pronouncing provider), the Nursing Supervisor, and the family per the patient's wishes.
3. Notify the OPO of the death/imminent death on the required timeline; do not approach the family about donation first.

4. Determine whether the death is a medical examiner / coroner case; if so, preserve lines, tubes, and devices and do not perform routine postmortem care until released.
5. Offer the family the opportunity to request an autopsy and provide information on how to do so.

5. Postmortem Care

- Provide culturally and religiously sensitive care of the body and support to the family; allow time at the bedside.
- For a medical examiner case, leave all medical devices in place and sequester the body as instructed until the case is released.
- Confirm identification of the body, complete required tags and paperwork, and coordinate transfer to the morgue or funeral home.
- Complete the death certificate per state requirements and within the required time.

6. Severity & Reporting

Expected deaths consistent with the natural course of illness are not reportable events. A death that is unexpected, or that may be related to an error, device, restraint or seclusion, or an unanticipated outcome, is a sentinel-level SEV-1 event under POL-RM-013 — notify the attending provider, Nursing Supervisor, Risk Management, and the Administrator-on-Call immediately and within 1 hour, and manage per POL-RM-004. Any death in or within 24 hours of restraint or seclusion is SEV-1 and additionally handled under POL-NUR-005. Unanticipated-outcome deaths are disclosed to the family under POL-PC-067.

7. Documentation Requirements

- Pronouncement provider, date, and time; examination findings supporting the determination.
- OPO notification time and reference number; medical examiner determination and release.
- Family notification and any autopsy request or declination.
- Disposition of the body and devices; death-certificate completion.

8. References

- CMS Conditions of Participation, 42 CFR 482.13 (patients' rights), 42 CFR 482.45 (organ, tissue, and eye procurement), and 42 CFR 482.22 (autopsy).
- The Joint Commission, standards on organ/tissue donation and care at the end of life.
- Related: POL-NUR-005 (restraint & seclusion), POL-PC-067 (disclosure), POL-RM-004 (sentinel events), POL-RM-013 (severity & reporting).